



Transthoracic Echocardiography Report

UNIT DIAGNOSTIK NON INVASIF
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Patient Information

Name : MULYADI MULYADI	Study Date : 30/01/2024	Height : 167cm	Diagnosis : Susp. Amyloidosis
MRN : 2024542485	DOB : 07/01/1969	Weight : 62kg	Referring physician : dr. Bambang Widiantoro, SpJP(K)
Age : 55	Gender : male	BSA : 1.7m²	Operator : GAT / RAR
First Write Time : 30/01/2024 21:01:48	Last Write Time : 01/02/2024 16:33:23	Blood Pressure : 90 / 73 mmHg	Medical Device : Vivid E95

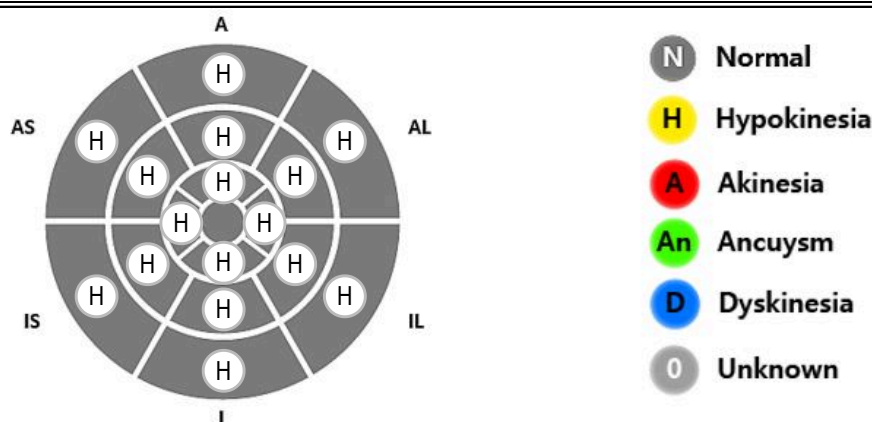
MMode / 2D Measurements & Calculations

IVSd : 17.6mm	Aor Diam : 21.6mm	EDV(teich) : 86.8ml	EF(Biplane) : 35%
IVSs : 21.6mm	LA dimen : 48.7mm	ESV(teich) : 46ml	EF(4ch) :
LVIDd : 43.8mm	LVOT diam : 21mm	EF : 48%	EF(2ch) :
LVIDs : 33.4mm	LA/Ao : 2.3mm	FS : 23.7%	IVC(Ins) :
LVPWd : 15mm	LAVI :	TAPSE : 9mm	IVC(EXP) :
LVPWs : 18.2mm	LV mass Index : 48g/m²		

Doppler Measurements & Calculations

MV E vel : 0.99m/sec	E/E'(medial) :	MV V max :	MV VTI :
MV DT : 116msec	E/E'(lateral) : 25	MV mean PG :	MV PHT :
MV A vel : 0.35m/sec	E/E'(average) : 31.4	MVA(plan) : cm²	MVA(VTI) :
E/A : 2.80			MVA by PHT :
AV V max :	AI PHT :	TR V max(systolic) : 2.91m/sec	
AV max PG :	AI(EROA) :	TR max PG(systolic) : 33.8mmHg	
AV mean PG :	LVOT VTI :	TV mean PG(diastolic) :	
AV VTI :	Descending Aorta(EDV) :	TV VTI :	
AVA (VTI) :	SV(LVOT) :	PV Acc time : 74msec	
AVA(Vmax) :	AVA(plan) : cm²		

2D Wall-Motion Diagram



Findings

ECG Rhythm	ECG is sinus rhythm.
Study Quality	The quality of the study is good.
Left Ventricle	Left ventricle size is normal. There is concentric left ventricular hypertrophy. RWT 0.68 ; LVMI 177 g/m2. Global left ventricular systolic function is reduced. LVEF 35% (Biplane) 30% (3DLVEF) LV diastolic dysfunction grade III. E/A ratio 2.8. av E/e 31 E' septal 3 cm/s ; E' lateral 3 cm/s. LV GLS is -6.3% (severe decreased). There is cherry on top appearance from speckle tracking LV evaluation (longitudinal strain of basal and mid segments which are severe hypokinesia and normokinetic apical segments). EFSR (ejection fraction strain ratio) = 5.5 ; RELAPS (Relative apical sparring) = 1.7 ; SAB (Septal to base ratio) = 6.
Right Ventricle	The right ventricle is normal in size. There is right ventricular hypertrophy. Right ventricular systolic function is decreased. TAPSE 0.9 ; RV FAC 15%.
Left Atrium	Left atrium is dilated. LAVI 48.1 mL/m2.
Right Atrium	Right atrium is dilated. RA area 18.9 cm2.
Interatrial Septum	There is stretched PFO bidirectional shunt. IAS thickness is 0.8 cm (from sub-xiphoid view).
Interventricular septum	Intact interventricular septum.
Aortic Valve	The aortic valve is trileaflet and opens well. There is no calcification. No aortic regurgitation is seen.
Mitral Valve	The mitral valve structure is normal. Mitral valve opens well. There is mild mitral regurgitation.
Tricuspid Valve	The tricuspid valve structure is normal. Tricuspid valve opens well. There is mild tricuspid regurgitation.

Pulmonic Valve	Pulmonic valve appears normal. There is no pulmonary regurgitation.
Mass/Thrombus	There is no abnormal mass or thrombus.
Pericardium	There is minimal pericardial effusion surrounding the cardiac (4 mm)
Others	Intermediate probability of pulmonary hypertension.

Conclusion

- Biventricular hypertrophy with reduced LV and RV systolic function. LVEF 35% (Biplane) with echo findings suggestive cardiac amyloidosis
- Global Hypokinetic
- Diastolic dysfunction grade III
- Mild MR
- Mild TR with intermediate probability of PH
- Minimal pericardial effusion

Comments

Operator : Fellow : Supervisor Confirm :

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Captured Patient Images

